

Media Information

From: Main Street Radiology To: Page: 1/2 Date: 9/5/2025 4:48:06 PM



Bayside 32nd Ave 32-35 Francis Lewis Blvd, Bayside, NY 11358
Bayside 44th Ave 44-01 Francis Lewis Blvd, Bayside, NY 11361
Downtown Flushing 136-25 37th Ave, Flushing, NY 11354
Western Queens 72-06 Northern Blvd, Jackson Hts, NY 11372
Glendale 83-14 Cooper Ave, Glendale, NY 11383

• HUI WEN HUANG, NP
• 135-27 38th Ave, 2nd FL
FLUSHING, NY

Referrer Phone:
Name: FENGMEI SUN
Phone:
Gender: Female
DOB: 3/4/1954
Patient:
Exam Date: 8/9/2025 10:13 AM

Acc #: 10036245
Secondary ID:
Exam Name:

Indication: Patient screening. The patient has no of breast cancer. The patient

Breast Cancer Risk:

NCI 5 year:0.6%
NCI Lifetime:2.0%

Last reported clinical breast exam: Does not remember

Comparison: Prior exams dating back to 2019.

Technique: Digital tomosynthesis CC and MLO views of both breasts were obtained. Reconstructed C views were also acquired. CAD was utilized. Deep learning algorithms (AI) were applied to mammography images and DBT slices as a concurrent reading aid.

Mammography:

The breast tissue is heterogeneously dense, which may obscure small masses.

There are no

Ultrasound:

Complete bilateral breast ultrasound including all 4 quadrants, retroareolar regions and the axillae.:

Right breast:

No

12:00 4 cm from nipple, a 0.6 cm hypoechoic mass is stable and benign.

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Continued...

Main Street Radiology

Name:

Date of Birth: 3/4/1954

11:00 9 cm from nipple, a 0.4 cm intramammary lymph node is identified.
11:00 3 cm from nipple, a 0.8 cm hypoechoic mass is stable and benign.
11:00 1 cm from nipple, minimal ectasia is noted.
12:00 retroareolar, a 0.5 cm septated cyst is noted.

Left breast:

There is no

1:00 2 cm from nipple a 0.3 cm complicated cyst is identified.
3:00 4 cm from nipple, a 0.5 cm hypoechoic mass with indistinct borders is noted.
6:00 retroareolar, a 0.5 cm hypoechoic mass is stable and benign.

IMPRESSION:

A 0.5 cm indeterminate mass is identified at 3:00 4 cm from nipple in the left breast, newly visualized. An ultrasound-guided core needle biopsy with post-procedure diagnostic mammogram is recommended for further evaluation.

Recommendation: Ultrasound Biopsy

BI-RADS CATEGORY: 4 - Suspicious Abnormality

A letter will be sent to the patient

Approximately 10% of breast carcinomas are not radiographically detectable. A negative report should not delay biopsy if a clinically suspicious mass is present. Dense breasts may obscure an underlying neoplasm.

Patient a target due date for their next mammogram.

BIRADS 1: Negative

BIRADS 2: Benign

BIRADS 3: Probably Benign

BIRADS 4: Suspicious Abnormality - Biopsy should be considered.

BIRADS 5: Highly Suspicious of Malignancy

BIRADS 6: Known Malignancy

BIRADS 0: Incomplete - Need Additional Imaging Evaluation and/or Prior Mammograms for Comparison

The New York State Department of Health requires us to indicate the date of the patient's last clinical breast examination.

If NCI risk has been calculated, the results are based on information provided by the patient.

Electronically signed by: Ron Benjamin MD 11:34 AM

Workstation: OUTREAD2

Report Electronically Signed by: Ron Benjamin MD
Report Electronically Signed on: 11:34 AM

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Document Information

Misc Administration: Outside Radiology, Breast

US

00:00

Attached To:

OUTSIDE RADIOLOGY ORDER

Orders Only on with Historical Provider,

Source Information

Rea Kate Toy | Msk Pas